300P

Zolbetuximab plus mFOLFOX6/CAPOX as first-line (1L) treatment in patients with claudin 18 isoform 2-positive (CLDN18.2+) metastatic gastric or gastroesophageal junction (mG/GEJ) adenocarcinoma with peritoneal metastases: A post hoc analysis of SPOTLIGHT and GLOW

Rui-hua Xu,[1] Tianshu Liu,[2] Jia Wei,[3] Dongsheng Zhang,[1] Rongrui Liu,[4] Qianyu Liu,[5] Na Meng,[5] Janise Lee,[6] Radhika Ranganath,[7] Jianming Xu[8]

[1]Sun Yat-Sen University Cancer Center, Guangzhou, China; [2]ZhongShan Hospital, FuDan University, Shanghai, China; [3]Nanjing Drum Tower Hospital, Nanjing, China; [4]The First Medical Center, Chinese PLA General Hospital, Beijing, China; [5]Astellas China Investment Co., Ltd, Beijing, China; [6]Astellas Pharma Singapore Pte. Ltd, Singapore; [7]Astellas Pharma Global Development, Northbrook, IL, USA; [8]The Fifth Medical Center, Chinese PLA General Hospital, Beijing, China

BACKGROUND

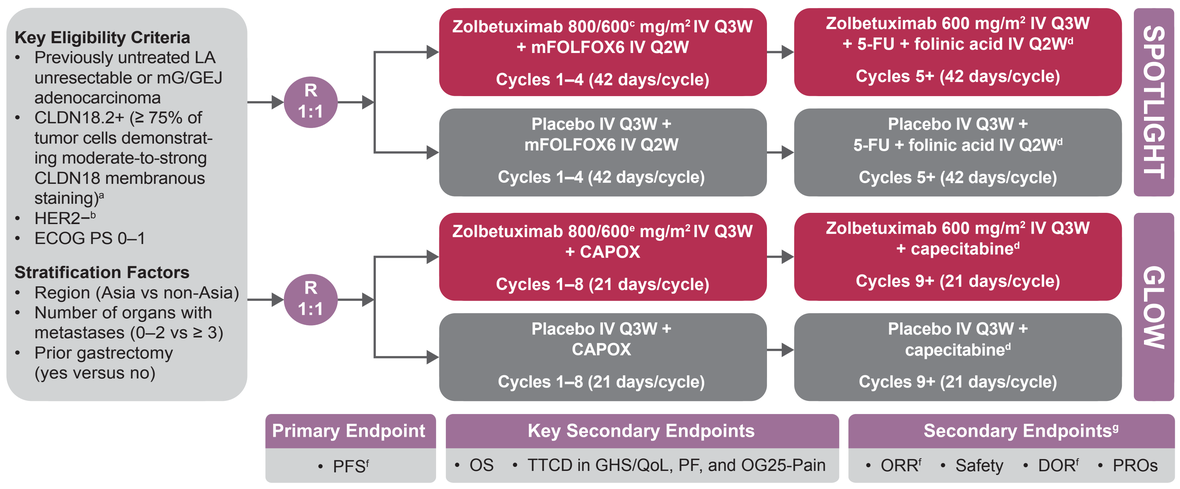
- Peritoneal metastases occur in approximately 25%–43% of patients with gastric cancer and are associated with poor prognosis[1,2]
- Despite therapeutic advances, efficacy of immunotherapies in patients with peritoneal metastases from gastric cancer remains uncertain mainly due to the unique clinical features of peritoneal carcinomatosis[3,4]
- The phase 3 SPOTLIGHT (NCT03504397) and GLOW (NCT03653507) trials showed statistically significant improvement in progression-free survival (PFS) and overall survival (OS) with 1L zolbetuximab plus modified fluorouracil, leucovorin or folinic acid, and oxaliplatin regimen (mFOLFOX6; SPOTLIGHT trial)/capecitabine and oxaliplatin (CAPOX; GLOW trial) versus placebo plus mFOLFOX6/CAPOX in patients with locally advanced (LA) unresectable or mG/GEJ adenocarcinoma[5,6]
- We assessed the efficacy of zolbetuximab plus chemotherapy versus placebo plus chemotherapy in patients with peritoneal metastases at baseline in SPOTLIGHT and GLOW

METHODS

Patients and treatment in SPOTLIGHT and GLOW

- Details of SPOTLIGHT, GLOW, and their combined analysis were published previously[5–7] (Figure 1)
- Adults with human epidermal growth factor receptor 2-negative (HER2–), previously untreated, LA unresectable or mG/GEJ adenocarcinoma whose tumors were CLDN18.2+, defined as ≥ 75% of tumor cells demonstrating moderate-to-strong membranous CLDN18 staining as determined by central immunohistochemistry using the VENTANA CLDN18 (43-14A) RxDx Assay (Roche Diagnostics Solutions, Tucson, AZ, USA), were included
- Patients received zolbetuximab 800 mg/m² (cycle [C] 1, day [D] 1) followed by 600 mg/m² (every 3 weeks [Q3W]) plus chemotherapy (mFOLFOX6 [SPOTLIGHT] or CAPOX [GLOW]), or placebo plus chemotherapy (mFOLFOX6 [SPOTLIGHT] or CAPOX [GLOW])
- In SPOTLIGHT, zolbetuximab was administered on D1 and D22 (Q3W) and mFOLFOX6 was administered on D1, D15, and D29 (every 2 weeks) of each 42-day treatment cycle. In GLOW, zolbetuximab was administered on D1 (Q3W), and CAPOX was administered on D1 to D14 of each 21-day treatment cycle
- In this post hoc analysis, patients with peritoneal metastases at baseline with and without liver metastases were included
  - Median (m) PFS and OS were estimated using the Kaplan–Meier method. Hazard ratios (HRs) with 95% confidence intervals (CIs) were calculated using Cox proportional hazards models
  - Safety was assessed as adverse events (AEs)

Figure 1. SPOTLIGHT and GLOW study designs



[a]By central IHC using the VENTANA CLDN18 (43-14A) RxDx Assay (Roche Diagnostics Solutions, Tucson, AZ, USA); [b]by central or local HER2 testing (IHC 0–1, or IHC 2/FISH–); [c]800 mg/m² at C1D1 followed by 600 mg/m² on day 1 of subsequent cycles; [d]at the discretion of the investigator; [e]800 mg/m² at C1D1 followed by 600 mg/m² on day 1 of subsequent cycles; [f]per RECIST version 1.1 by independent review committee; [g]not all secondary endpoints are listed.

5-FU, 5-fluorouracil; C, cycle; CAPOX, capecitabine and oxaliplatin; CLDN18.2+, claudin 18 isoform 2-positive; D, day; DOR, duration of response; ECOG PS, Eastern Cooperative Oncology Group performance status; FISH, fluorescence in situ hybridization; GHS, global health status; HER2–, human epidermal growth factor receptor 2-negative; IHC, immunohistochemistry; IV, intravenous; LA, locally advanced; mFOLFOX6, modified folinic acid + fluorouracil + oxaliplatin regimen; mG/GEJ, metastatic gastric or gastroesophageal junction; OG25, Quality of Life of Cancer Patients questionnaire, Oesophago-Gastric module; ORR, objective response rate; OS, overall survival; PF, physical functioning; PFS, progression-free survival; PROs, patient-reported outcomes; Q2W, every 2 weeks; Q3W, every 3 weeks; QoL, quality of life; R, randomized; RECIST v1.1, Response Evaluation Criteria in Solid Tumors version 1.1; TTCD, time to confirmed deterioration.

RESULTS

Patients

- In total, 1072 patients across SPOTLIGHT and GLOW were randomly assigned to the zolbetuximab arm (n = 537) or the placebo arm (n = 535)
  - Of these, 34.9% (374/1072) had peritoneal metastases at baseline and were included in this post hoc analysis (zolbetuximab arm, n = 192; placebo arm, n = 182)
  - Baseline demographics and clinical characteristics were balanced between treatment arms (Table 1)

Table 1. Baseline demographics and clinical characteristics in patients with peritoneal metastases; full analysis set

		Zolbetuximab + chemotherapy (n = 192)	Placebo + chemotherapy (n = 182)
Age, years	Median (range)	59 (22.0–82.0)	57 (20.0–79.0)
Sex, n (%)	Male	103 (53.6)	97 (53.3)
	Female	89 (46.4)	85 (46.7)
Race, n (%)	White	81 (44.3)	72 (41.1)
	Black or African American	1 (0.5)	1 (0.6)
	Asian	92 (50.3)	91 (52.0)
	American Indian or Alaska Native	4 (2.2)	2 (1.1)
	Others[a]	5 (2.7)	9 (5.1)
	Missing	9	7
Organs with concomitant metastases, n (%)	Abdominal cavity	5 (2.6)	6 (3.3)
	Bone	11 (5.7)	12 (6.6)
	Liver	31 (16.1)	24 (13.2)
	Lung	15 (7.8)	14 (7.7)
	Ovary	24 (12.5)	20 (11.0)
	Other	10 (5.2)	16 (8.8)
Primary site, n (%)	Stomach	171 (89.1)	167 (91.8)
	GEJ	21 (10.9)	15 (8.2)
Lauren classification, n (%)	Diffuse	84 (44.2)	89 (49.4)
	Intestinal	23 (12.1)	25 (13.9)
	Mixed	13 (6.8)	12 (6.7)
	Others[b]	70 (36.8)	54 (30.0)
	Missing	2	2
ECOG PS, n (%)	0	78 (41.5)	78 (43.6)
	1	110 (58.5)	101 (56.4)
	Missing	4	3

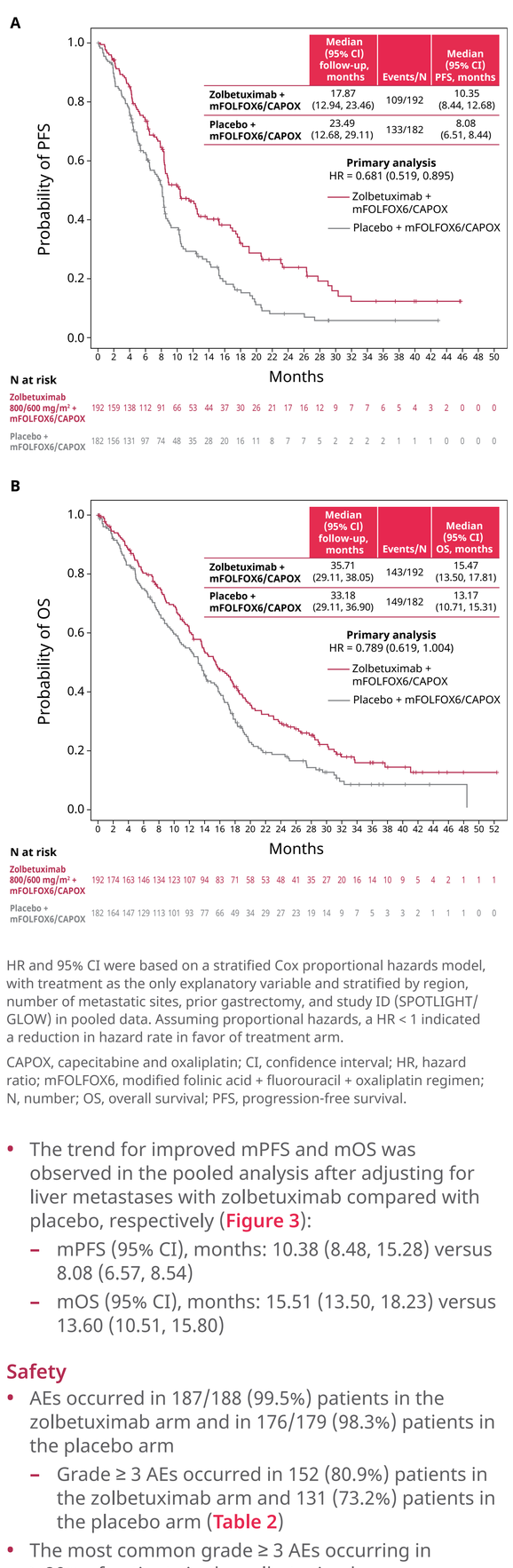
[a]Others in "Race" include Native Hawaiian or Other Pacific Islander, and Other; [b]included patients whose tumors were classified as unknown, or other types.

ECOG PS, Eastern Cooperative Oncology Group performance status; GEJ, gastroesophageal junction.

Efficacy

- In the pooled analysis, patients with peritoneal metastases showed a trend for improved mPFS and mOS with zolbetuximab compared with placebo, respectively (Figure 2):
  - mPFS (95% CI), months: 10.35 (8.44, 12.68) versus 8.08 (6.51, 8.44)
  - mOS (95% CI), months: 15.47 (13.50, 17.81) versus 13.17 (10.71, 15.31)

Figure 2. PFS (A) and OS (B) in patients with peritoneal metastases; full analysis set



HR and 95% CI were based on a stratified Cox proportional hazards model, with treatment as the only explanatory variable and stratified by region, number of metastatic sites, prior gastrectomy, and study ID (SPOTLIGHT/GLOW) in pooled data. Assuming proportional hazards, a HR < 1 indicated a reduction in hazard rate in favor of treatment arm.

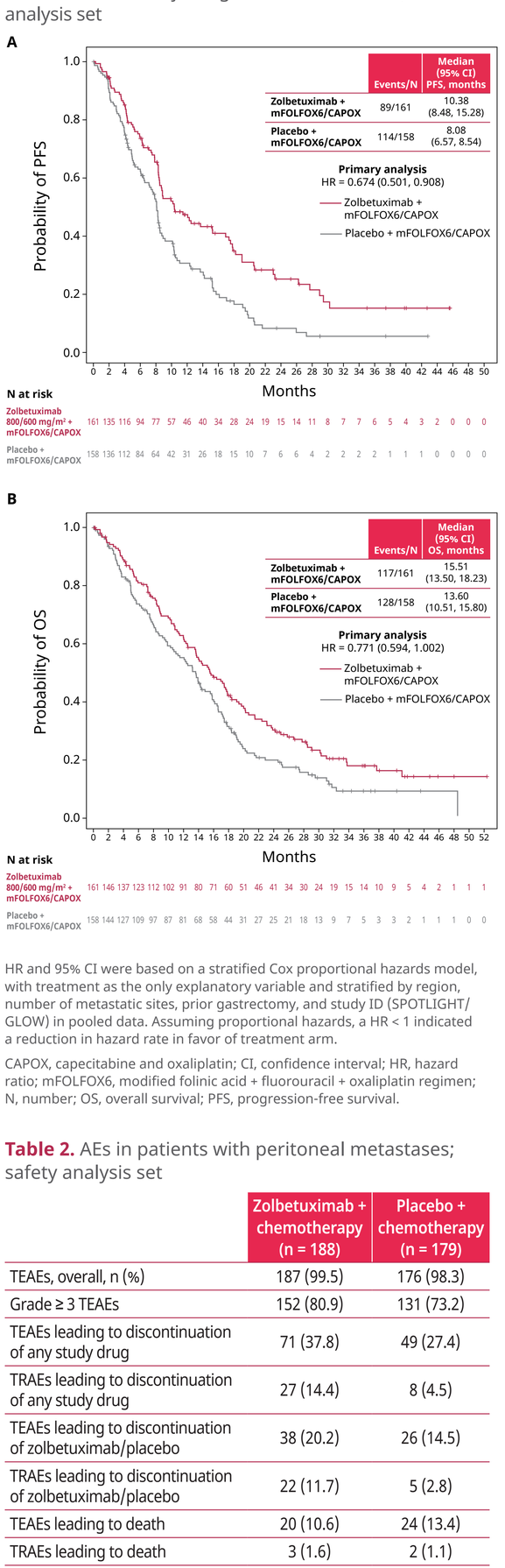
CAPOX, capecitabine and oxaliplatin; CI, confidence interval; HR, hazard ratio; mFOLFOX6, modified folinic acid + fluorouracil + oxaliplatin regimen; N, number; OS, overall survival; PFS, progression-free survival.

- The trend for improved mPFS and mOS was observed in the pooled analysis after adjusting for liver metastases with zolbetuximab compared with placebo, respectively (Figure 3):
  - mPFS (95% CI), months: 10.38 (8.48, 15.28) versus 8.08 (6.57, 8.54)
  - mOS (95% CI), months: 15.51 (13.50, 18.23) versus 13.60 (10.51, 15.80)

Safety

- AEs occurred in 187/188 (99.5%) patients in the zolbetuximab arm and in 176/179 (98.3%) patients in the placebo arm
  - Grade ≥ 3 AEs occurred in 152 (80.9%) patients in the zolbetuximab arm and 131 (73.2%) patients in the placebo arm (Table 2)
- The most common grade ≥ 3 AEs occurring in ≥ 20% of patients in the zolbetuximab arm were neutropenia (20.7%) and vomiting (20.2%) (Figure 4)

Figure 3. PFS (A) and OS (B) in patients with peritoneal metastases after adjusting for liver metastases; full analysis set



HR and 95% CI were based on a stratified Cox proportional hazards model, with treatment as the only explanatory variable and stratified by region, number of metastatic sites, prior gastrectomy, and study ID (SPOTLIGHT/GLOW) in pooled data. Assuming proportional hazards, a HR < 1 indicated a reduction in hazard rate in favor of treatment arm.

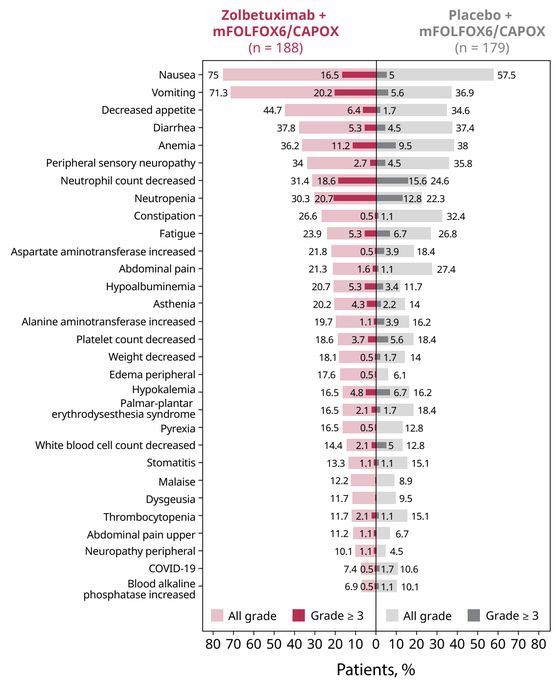
CAPOX, capecitabine and oxaliplatin; CI, confidence interval; HR, hazard ratio; mFOLFOX6, modified folinic acid + fluorouracil + oxaliplatin regimen; N, number; OS, overall survival; PFS, progression-free survival.

Table 2. AEs in patients with peritoneal metastases; safety analysis set

	Zolbetuximab + chemotherapy (n = 188)	Placebo + chemotherapy (n = 179)
TEAEs, overall, n (%)	187 (99.5)	176 (98.3)
Grade ≥ 3 TEAEs	152 (80.9)	131 (73.2)
TEAEs leading to discontinuation of any study drug	71 (37.8)	49 (27.4)
TRAEs leading to discontinuation of any study drug	27 (14.4)	8 (4.5)
TEAEs leading to discontinuation of zolbetuximab/placebo	38 (20.2)	26 (14.5)
TRAEs leading to discontinuation of zolbetuximab/placebo	22 (11.7)	5 (2.8)
TEAEs leading to death	20 (10.6)	24 (13.4)
TRAEs leading to death	3 (1.6)	2 (1.1)

TEAE, treatment-emergent adverse event; TRAE, treatment-related adverse event.

Figure 4. TEAEs that occurred in ≥ 10% of patients with peritoneal metastases; safety analysis set



TEAEs occurring in ≥ 10% of patients in either treatment arm were reported.

CAPOX, capecitabine and oxaliplatin; mFOLFOX6, modified folinic acid + fluorouracil + oxaliplatin regimen; TEAE, treatment-emergent adverse event.

CONCLUSIONS

- This post hoc analysis in patients with baseline peritoneal metastases indicates that 1L zolbetuximab plus chemotherapy provides numerically greater mPFS and preserves mOS compared with chemotherapy alone (versus overall population), demonstrating that the magnitude of benefit is not diminished in this poor-prognosis subgroup, who currently lack evidence-based, effective targeted therapy options
- Safety events for patients in this post hoc analysis were similar to those reported in SPOTLIGHT, GLOW, or the pooled intent-to-treat population
- In conclusion, zolbetuximab plus chemotherapy should be considered upfront for all patients with HER2–, CLDN18.2+, LA unresectable or mG/GEJ adenocarcinoma irrespective of peritoneal metastases

References

1. de Moraes FCA et al. *J Gastrointest Cancer*. 2025;56(1):206. 2. Sirody J et al. *Am J Surg*. 2022;224(1 Pt B):445-48. 3. Ornella MSC et al. *Cancers (Basel)*. 2023;15(8):2383. 4. Zhou Y et al. *NPJ Precis Oncol*. 2025;9(1):247. 5. Shitara K et al. *Lancet*. 2023;401(10389):1655-68. 6. Shah MA et al. *Nat Med*. 2023;29(8):2133-41. 7. Shitara K et al. *N Engl J Med*. 2024;391(12):1159-62.

Disclosures

Tianshu Liu: none

Acknowledgments

The authors thank the patients, caregivers, investigators, and site staff who participated in this study. Medical writing support, conducted in accordance with Good Publication Practice (GPP 2022) and the International Committee of Medical Journal Editors (ICMJE) guidelines, was provided by Priyanka D. Pinky, PhD, of Oxford PharmaGenesis Inc., Wilmington, DE, USA, and funded by Astellas Pharma Inc.

Funding

Study sponsored by Astellas Pharma Inc.

For more information please contact:
Prof. Tianshu Liu, liu.tianshu@zs-hospital.sh.cn.

Click or scan this quick response (QR) code to download this poster, along with associated material.

Copies of this poster obtained through QR, AR and/or text key codes are for personal use only and may not be reproduced without written permission of the authors.

Presented at the European Society for Medical Oncology (ESMO) Asia Congress 2025; 5–7 December 2025; Singapore